

D223

Healthcare Policy and Economics

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A clinic with a popular new website is paid fee-for-service. Its administrator notices that repeat visits for poorly controlled diabetes raise revenue. The clinic has four physicians. Which incentive does fee-for-service create?

- A. More services bring more revenue, regardless of patient outcomes.
- B. Better outcomes bring higher payments.
- C. A fixed payment per member regardless of visits.
- D. Fee-for-service never creates any incentive to overtreat.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

Under a value-based contract, a clinic earns a bonus if its diabetic patients' A1c control improves while total costs stay below target. Which behavior does this reward?

- A. Cutting services to save money even if A1c control worsens.
- B. Keeping patients healthy through prevention and care coordination.
- C. Ordering every available test for each patient.
- D. Scheduling more visits to raise billing.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A hospital with a new parking garage accepts a single \$25,000 bundled payment for a hip replacement episode covering surgery, the hospital stay, and 90 days of rehab. Its surgeons are nationally ranked. If the episode actually costs \$22,000, what happens?

- A. The hospital bills the insurer an additional \$22,000.
- B. The hospital keeps the \$3,000 difference.
- C. The hospital receives \$47,000.
- D. The hospital loses \$3,000.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A primary care group in a fast-growing suburb receives \$50 per member per month for 2,000 enrolled patients. Its lead physician trained abroad. What is the monthly revenue, and how does it change if patients visit more often?

- A. \$100,000; it does not change with visit volume.
- B. \$2,050 per month
- C. \$1,200,000 per month
- D. \$100,000, and it rises with more visits.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5. [TRANSFER — reversed stem / different variable]

Under capitation, who bears the financial risk if enrolled patients need more care than expected?

- A. The provider group
- B. The patient, through higher copays
- C. No one, because the payment is fixed
- D. The insurer

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A 60-year-old with stable, mild knee arthritis at a clinic with a new MRI machine is choosing between physical therapy and knee replacement. Both are reasonable, and the best choice depends on how she weighs surgical risk against recovery time. What approach fits?

- A. Let the surgeon decide, since surgeons know best.
- B. Let the MRI result decide the right answer.
- C. Shared decision-making, using a decision aid that reflects her values.
- D. Choose the cheaper option automatically.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A 67-year-old retired truck driver who lives in a rural county and has no disability enrolls in public coverage based on his age. His grandson plays college football. Which program is it?

- A. CHIP
- B. Medicaid
- C. Medicare
- D. Medicaid, since all rural residents qualify

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A patient with a plan through her employer, a regional bank, has a \$1,000 deductible (not yet met) and 20% coinsurance. She has outpatient surgery with an allowed amount of \$6,000. Ignoring the out-of-pocket maximum, what does she pay?

- A. \$1,200
- B. \$2,200
- C. \$2,000
- D. \$1,000

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9. [TRANSFER — reversed stem / different variable]

A patient paid \$1,500 in total on a \$5,500 allowed charge. Her \$500 deductible had not been met, and the rest of what she paid was coinsurance. What is her coinsurance rate?

- A. About 27%
- B. About 18%
- C. About 9%
- D. 20%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A hospital with a new rooftop garden has higher-than-expected 30-day readmission rates for heart failure. Its CEO recently spoke at a national conference. Under Medicare's Hospital Readmissions Reduction Program, what happens?

- A.** Readmitted patients pay the penalty through higher copays.
- B.** Medicare reduces the hospital's base payments.
- C.** The hospital automatically loses Medicare certification.
- D.** Medicare pays a bonus for each readmission.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

An ACO with 20,000 attributed Medicare patients and a new analytics dashboard has a spending benchmark of \$200 million. Actual spending is \$190 million, and it meets quality standards. With a 50% shared-savings rate, how much does the ACO earn?

- A.** \$95 million
- B.** \$10 million
- C.** \$100 million
- D.** \$5 million

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

An ACO earned \$3 million in shared savings at a 60% sharing rate. What were total savings below the benchmark?

- A.** \$3 million
- B.** \$7.5 million
- C.** \$1.8 million
- D.** \$5 million

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer A Concept 1

A clinic ~~with a popular new website~~ is paid fee-for-service. Its administrator notices that repeat visits for poorly controlled diabetes raise revenue. ~~The clinic has four physicians.~~ Which incentive does fee-for-service create?

Target word: FFS incentive **Decides:** Payment is per service.

A	Correct	FFS rewards volume.
B	Inverted operation	That describes value-based payment.
C	Confusion set	That is capitation.
D	Absolute language	Paying per service can encourage more services.

2. Answer B Concept 1 • transfer

Under a value-based contract, a clinic earns a bonus if its diabetic patients' A1c control improves while total costs stay below target. Which behavior does this reward?

Target word: behavior rewarded **Decides:** Pay depends on outcomes and cost together.

A	Dropped element	Drops the quality half of the contract.
B	Correct	Better control at lower cost earns the bonus.
C	Adjacent category	Raises cost without necessarily improving outcomes.
D	Inverted operation	That is the fee-for-service incentive.

3. Answer B Concept 2

A hospital ~~with a new parking garage~~ accepts a single \$25,000 bundled payment for a hip replacement episode covering surgery, the hospital stay, and 90 days of rehab. ~~Its surgeons are nationally ranked.~~ If the episode actually costs \$22,000, what happens?

Target word: result under a bundle **Decides:** The fixed payment exceeds actual cost by \$3,000.

A	Adjacent category	That is fee-for-service thinking; the bundle covers the episode.
B	Correct	$\$25,000 - \$22,000 = \$3,000$ retained.
C	Wrong stem number	Adds the payment and the cost.
D	Inverted operation	Costs came in under the payment, not over.

4. Answer A Concept 3

A primary care group ~~in a fast-growing suburb~~ receives \$50 per member per month for 2,000 enrolled patients. ~~Its lead physician trained abroad.~~ What is the monthly revenue, and how does it change if patients visit more often?

Target word: capitation revenue and behavior **Decides:** PMPM × members, fixed regardless of visits.

A	Correct	$\$50 \times 2,000 = \$100,000$, paid regardless of use.
B	Inverted operation	Adds the rate and the members instead of multiplying.
C	Confusion set	That is the annual amount.
D	Adjacent category	Visit-based revenue is fee-for-service.

5. Answer A Concept 3 · transfer

Under capitation, who bears the financial risk if enrolled patients need more care than expected?

Target word: who holds risk **Decides:** Payment is fixed while costs vary.

A	Correct	It must deliver the extra care for the same fixed payment.
B	Adjacent category	Capitation is a provider payment method, not patient cost sharing.
C	Dropped element	Ignores that costs can exceed the fixed payment.
D	Inverted operation	The insurer shifted utilization risk to the provider.

6. Answer C Concept 4

A 60-year-old with stable, mild knee arthritis at a clinic ~~with a new MRI machine~~ is choosing between physical therapy and knee replacement. Both are reasonable, and the best choice depends on how she weighs surgical risk against recovery time. What approach fits?

Target word: approach to a preference-sensitive decision **Decides:** Two reasonable options with value-dependent trade-offs.

A	Inverted operation	Removes the patient's values from the choice.
B	Camouflage	Imaging does not settle a values-based choice.
C	Correct	Preference-sensitive care follows the informed patient's priorities.
D	Adjacent category	Cost matters but does not replace her preferences.

7. Answer C Concept 5

A 67-year-old retired truck driver ~~who lives in a rural county~~ and has no disability enrolls in public coverage based on his age. ~~His grandson plays college football.~~ Which program is it?

Target word: program by eligibility **Decides:** Age 65 or older.

A	Adjacent category	CHIP covers children.
B	Confusion set	Medicaid eligibility is based on low income.
C	Correct	Federal coverage for people 65 and older.
D	Absolute language	Where he lives does not set eligibility.

8. Answer C Concept 6

A patient with a plan ~~through her employer, a regional bank,~~ has a \$1,000 deductible (not yet met) and 20% coinsurance. She has outpatient surgery with an allowed amount of \$6,000. Ignoring the out-of-pocket maximum, what does she pay?

Target word: patient cost **Decides:** Deductible first, then 20% of the remaining \$5,000.

A	Dropped element	20% of \$6,000 ignores the deductible.
B	Inverted operation	Applies coinsurance to the full \$6,000 after also charging the deductible.
C	Correct	$\$1,000 + 20\% \times \$5,000 = \$2,000$.
D	Wrong stem number	Only the deductible.

9. Answer D Concept 6 · transfer

A patient paid \$1,500 in total on a \$5,500 allowed charge. Her \$500 deductible had not been met, and the rest of what she paid was coinsurance. What is her coinsurance rate?

Target word: solve for coinsurance rate **Decides:** $(\$1,500 - \$500) \div (\$5,500 - \$500)$.

A	Dropped element	$\$1,500 \div \$5,500$ ignores the deductible.
B	Inverted operation	Removes the deductible from her payment but not from the charge.
C	Wrong stem number	$\$500 \div \$5,500$ uses the deductible as the coinsurance.
D	Correct	$\$1,000 \div \$5,000 = 20\%$.

10. Answer B Concept 7

A hospital ~~with a new rooftop garden~~ has higher-than-expected 30-day readmission rates for heart failure. ~~Its CEO recently spoke at a national conference.~~ Under Medicare's Hospital Readmissions Reduction Program, what happens?

Target word: HRRP consequence **Decides:** Excess readmissions trigger payment reductions.

A	Adjacent category	The penalty falls on the hospital.
B	Correct	HRRP penalizes excess readmissions through reduced payments.
C	Absolute language	The penalty is a payment cut, not decertification.
D	Inverted operation	Readmissions are penalized, not rewarded.

11. Answer D Concept 8

An ACO ~~with 20,000 attributed Medicare patients and a new analytics dashboard~~ has a spending benchmark of \$200 million. Actual spending is \$190 million, and it meets quality standards. With a 50% shared-savings rate, how much does the ACO earn?

Target word: shared savings earned **Decides:** $(\$200M - \$190M) \times 50\%$.

A	Inverted operation	Applies the rate to actual spending instead of savings.
B	Dropped element	Forgets the 50% sharing rate.
C	Wrong stem number	Applies the rate to the benchmark.
D	Correct	$\$10 \text{ million savings} \times 50\% = \5 million .

12. Answer D Concept 8 · transfer

An ACO earned \$3 million in shared savings at a 60% sharing rate. What were total savings below the benchmark?

Target word: solve for total savings **Decides:** $\$3M \div 0.60$.

A	Wrong stem number	That is the ACO's share, not total savings.
B	Adjacent category	Divides by the payer's 40% share instead of the ACO's 60%.
C	Inverted operation	Multiplies by 0.60 instead of dividing.
D	Correct	$\$3 \text{ million} \div 0.60 = \5 million .

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	5	
Wrong stem number	A figure copied from the prompt that is not the answer.	5	
Adjacent category	The right idea in the wrong bucket.	8	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	11	
Absolute language	Always, never, guarantee — where judgment applies.	3	
Confusion set	Every option is real; the test is exact role-matching.	3	
Camouflage	Answers a stem fact that was never the question.	1	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Fee-for-service vs. value-based incentives	Fee-for-service pays per service, rewarding volume; value-based payment rewards outcomes and cost control.	Assuming fee-for-service rewards better outcomes.	What the provider is paid for.
2	Bundled payments	One fixed payment covers an entire episode of care; the provider keeps savings below it and absorbs costs above it.	Thinking providers can bill extra services on top of a bundle.	A single price for surgery plus follow-up care.
3	Capitation	A fixed amount per member per month regardless of how much care is used; the provider bears utilization risk.	Thinking revenue rises with more visits.	"Per member per month."
4	Preference-sensitive care	When several reasonable options exist, the choice should follow the informed patient's values through shared decision-making.	Letting the clinician, cost, or a test result decide alone.	Two reasonable options with different trade-offs.
5	Medicare vs. Medicaid	Medicare is federal coverage mainly for people 65+ or with certain disabilities; Medicaid is federal-state coverage based on low income.	Mixing up the eligibility bases.	Age vs. income.
6	Patient cost sharing	The patient pays the deductible first, then coinsurance as a percentage of the remaining allowed amount.	Applying coinsurance to the full bill or ignoring the deductible.	A deductible, a coinsurance rate, and an allowed amount.
7	Readmission penalties	Medicare's Hospital Readmissions Reduction Program reduces payments to hospitals with excess 30-day readmissions for targeted conditions.	Thinking readmissions earn extra pay or lead to decertification.	Higher-than-expected readmissions.
8	ACO shared savings	Savings = benchmark – actual spending; the ACO earns its sharing rate times savings if it meets quality standards.	Forgetting to apply the sharing rate, or applying it to total spending.	A benchmark, actual spending, and a sharing percentage.

Course scope (public description): Healthcare Policy and Economics is a foundational course that introduces the concepts of value-based care and the role of the nurse. This course includes concepts related to financial responsiveness, shared decision-making, preference-sensitive care, leveraging data. In this course, students learn about cost and fee-for-service in terms of value to the client and patient rather than value to the healthcare system. All prior courses in the sequence for this program serve as prerequisites for this course.